



PROCEDURE

Wound Management

Scope (Staff):	All NMHS Mental Health (MH) Inpatient Nursing Staff
Scope (Area):	All NMHS MH Inpatient Services

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1. Aim

This procedure outlines the procedures and responsibilities of individuals and the health service in relation to Wound Management. It ensures the delivery of a multidisciplinary approach to wound management that is in accordance with [Australian Guidelines for the Prevention and Control of Infection in Healthcare \(2019\)](#) and the [Australian Healthcare Standards](#).

2. Risk

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

3. Key Points

Observe the five moments for hand hygiene. Follow aseptic technique when performing wound care and use the risk assessment outlined in the Policy. This must include use of appropriate Personal Protective Equipment (PPE) to prevent contamination and mucosal or conjunctival splash injuries [MHPHDS Aseptic Technique Policy](#).

4. Equipment

- Standard Dressing Trolley
- Sterile dressing packs
- Prescribed cleaning solution

5. Processes

5.1 General Management

- All patients require an initial physical assessment to be completed within 12 hours of admission by a Medical Officer. This includes completion of a Physical Examination (SMHMR903) and a Physical Appearance (TMHMR904) including a full skin assessment noting any abnormalities or wounds requiring further investigation and/or tests to be completed within 7 days of admission in accordance with [Statewide Standardised Clinical Documentation \(SSCD\)](#).
- Certain topical procedures require prescription (e.g. Silver Sulphadiazine cream (SSC), and Metronidazole gel).
- If medical photography is required, refer to the SCGH Clinical Photography Policy and the Star Skin Tear Classification System (**Appendix 1**)
- Documentation on the Wound Management Plus must include:
 - wound type
 - location

- dimensions
- appearance
- exudate
- The Wound Management Plan should be discussed with the patient, family and carer and documented in the progress notes. Refer to the Braden Risk Assessment and Pressure Injury Management plan if indicated (PMR 9) (**Appendix 2**).
- Wound and dressing management including outcome of all assessments, interventions and clinical reviews is to be accurately and legibly recorded in the medical records.
- Assessment of pain specific should be undertaken prior to performing a change of dressing and use of analgesia as indicated. Consider use of the Abbey Pain Assessment Scale for Older Adult cohort with administration of analgesia as indicated / prescribed (**Appendix 3**).

5.2 Factors that may impair wound healing (Table 1)

Consider the following factors listed below in **Table 1** that may impair wound healing

Intrinsic factors	Extrinsic factors	Local factors
• Malnutrition Obesity • Psycho–social well-being • Diabetes • Arterial disease - Peripheral Vascular Disease (PVD) • Advanced age • Disease Process	• Smoking, alcohol abuse • Medications • Radiotherapy, Chemotherapy • Temperature	• Desiccation & Maceration of the wound bed, • Infection, • Pressure, shear and friction, • Foreign body, • Dressing selection, • Chemical stress, • Wound pain

5.3 Wound Cleansing

The primary objective of wound cleansing is to remove debris, previous dressing material and foreign bodies from the wound bed.

Normal saline is the recommended cleansing agent to be used.

5.4 Acute Wounds

Acute wounds are surgical or traumatic wounds that heal in a progressive and timely manner within 4- 6-week period.

5.5 Chronic Non-Healing Wounds

Chronic Non-Healing Wounds (e.g. leg ulcer) are unpredictable in terms of healing progression and timeframes involved and require specialist assessment and management planning.

5.6 Documentation Requirements

- Document any change in the condition of wound.
- Document the patients wound and dressing management including outcome of all assessments, interventions and specific site wound management is to be accurately and legibly recorded in the patients' health records.
- Patient education should be given in the form of verbal and written information.

5.7 Discharge Planning

- Where patients require a continuation of management of their wound referral to the appropriate clinical services on discharge should be performed.
- Ensure referral and current wound management plan are supplied to the service provider.
- Provide education and a current wound management plan to the patient and or carer.
- Provide additional wound supplies on discharge.
- Ensure the patient/carer are aware of any follow-up appointments and/or service referrals.
- Provide education to the patient/carer regarding adverse signs and symptoms:
 - Infection
 - Wound deterioration, or other potential problems, e.g. pain or bleeding.

6. Skin Tear Prevention and Management

6.1 Medical Requirements

The Medical team is required to review the patient's immunisation status.

6.2 Safety Information

Only soft silicone dressings should be used as a primary dressing on skin tears (e.g. Mepitel or Mepilex).

The following adhesive dressings and tapes **should not** be used on those who are susceptible to skin tears due to fragility and high risk of further skin injury:

- Fixation tapes (e.g. Fixamull), skin closure (e.g. steri strips)
- Film dressings (e.g. Tegaderm)
- Hydrocolloid dressings (e.g. Duoderm)
- Adhesive Tapes or adherent dressings that may adhere and cause trauma.

6.3 Nursing Management

Use the **STAR Skin Tear Classification System** to categorise the type of Skin Tear (**Appendix 1**)

A skin tear can be:

- partial thickness (separation of the epidermis from the dermis); or
- full thickness (separation of both the epidermis from the dermis underlying structures)
- all large full thickness tears must be reported to the admitting medical team and referred to the Plastic's team for assessment and management.

Any skin tear acquired via iatrogenic causes must be reported using the Datix Clinical Incident Management System (CIMS).

Patients at risk of skin tear and development should have appropriate interventions implemented.

Adhesive skin closures (steri strips) **should not be used** at any time on Skin Tears, as this creates traction/erosion of skin and underlying issues. Suturing and stapling of a skin flap is not current best practice.



6.4 Risk Assessment

In addition to assessment on admission, patients should be assessed at each episode of care to identify their risk of sustaining a skin injury.

Perform a comprehensive assessment of the skin noting any of the factors identified in **Table 2** below.

Factors that increase risk of sustaining a skin tear (Table 2)

If one or more of factors listed below are present the patient is at risk of sustaining a skin tear • Extremes of age • Fragile skin • Previous skin tears General health factors: • Chronic/critical disease • Polypharmacy • Impairment: cognitive, sensory, visual, auditory, nutritional Mobility • History of falls • Impaired mobility • Dependent activities of daily living • Mechanical trauma	If any of the factors listed below are present the patient has an increased to high risk of sustaining a skin tear in the hospital environment • Decreased activity, mobility or sensory changes related to changes in health status / medications • Skin dry with presence of ecchymosis – bruising and discoloration of skin • Nutritional status is > 2 on Malnutrition Screening Tool • Dehydration and decreased skin turgor • At risk of pressure /friction /shearing of their skin. • Braden score < 18 • Patient that is cognitively impaired and in an unfamiliar environment
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6.5 Preventative Strategies

All patients at risk of injury should have standard interventions identified in the Management Plan as well as patient specific interventions.

Patients with mental state deterioration (agitation, confusion, delirium, mania) requiring physical guidance and/or restraining to prevent injury to self will require additional interventions document in the Management Plan to identify preventative strategies.

Preventative Strategies (Table 3)

Appropriate Skin Care	• Use natural pH skin Cleansers – avoid soap • Encourage BD skin moisturising with a neutral PH moisturiser
Safe Environment	• Encourage clothes, which are not restrictive, loose long sleeves and long trousers. • Ensure safe environment – no equipment with sharp edges. Foam padding may be required. • Encourage patients to wear their glasses, hearing aids and appropriate non-slip footwear. • Ensure clinical staff and patients avoid wearing jewellery, watches and maintain shortnails.
Injury Prevention	• Encourage patient mobilisation with care – communication and plan prior to patient movement. • Correct manual handling technique to prevent friction and shear when turning, lifting and transferring. Use of slide sheets, hover mat and other approved aids. • Prevent falls injuries. Undertake mobility and falls risk assessment and implement falls risk strategies. • Protect patients' skin and limbs from skin injury – consider extra padding of limbs e.g. foam held in situ with tubular band when indicated • Provide patient education to patients regarding promoting healthy skin e.g. ceasing smoking, preventing friction and shear • Discuss cause and risk of sustaining a skin injury with your patient/ family/carer and prevention and management plan.
Clinical Considerations	• Optimise nutrition and hydration. Refer to Dietician When Nutritional score >2 on Malnutrition Screening Tool (MST). • Review long-term use of medications known to alter skin integrity e.g. long-term use of steroid ointments. • Adhesive dressings and tapes not to be used at all. • Use of soft silicone tapes e.g. 3M (easy remove tape). • Soft silicone wound products only to be used when dressing skin tears e.g. Mepitel or Mepilex foam border. • Prophylactic dressing to prevent shearing and friction over high-risk areas

6.6 Presence of Skin Tear (ST)

1. Mark the Skin Tear (ST) on the diagram on the front of the Wound Management Plan.
2. Skin Tear Category should be classified using the Star Skin Tear Classification Guide (**Appendix 1**).
3. Assessment of skin tear wound appearance should be documented in the integrated notes:
 - Location of the skin tear
 - Size of skin tear:
 - Small < 5x5cm

- Medium > 5x5cm
 - Large > 10x10cm
 - Presence of skin flap and appearance
 - Identify if Skin Tear is partial thickness or full thickness
 - Presence of pain
 - Exudate amount, type and consistency
 - Peri-wound skin appearance
 - Initiate a Wound Management Plan
4. Skin tear acquired in hospital must be reported using Clinical Incident Management System (Hospital Acquired Complication).
 5. Ensure communication with interdisciplinary health team so they are aware of skin integrity issues identified and prevention/ treatment strategies in place.
 6. Medical review is required for full thickness and large skin tear and subsequent referral to Specialist and or Plastic's Registrar as required.
 7. Referral to Wound Management Specialist Nurse is required for those sustaining large skin tears or at any time wound management advice is required (SCGOPCG Wound Management).
 8. Monitor and record the effect of interventions implemented.
 9. Commence and document daily skin and Braden Scale for risk assessments.
 10. The patients wound and dressing management including outcome of all assessments, interventions and specific site wound management is to be accurately and legibly recorded in the patients' health record.

6.7 Aseptic Technique Wound Care (Uncomplicated)

1. Administer analgesia if required.
2. Perform hand hygiene.
3. Clean the trolley with a combined detergent/ disinfectant wipe.
4. Gather equipment for the procedure.
5. Perform hand hygiene.
6. Open dressing pack using the corners and add items required using a non-touch technique. Perform hand hygiene and put on non-sterile gloves.
7. Remove soiled dressing using a non-touch technique, (If the dressing is adhered then irrigating with saline in a 10ml syringe may be required to assist with removal and discard. Remove gloves.
8. Perform hand hygiene and put on non-sterile gloves.
9. Cleanse the skin tear with gauze and normal saline. Using forceps remove any debris or residual haematoma and gently irrigating with normal saline in a syringe to ensure area thoroughly cleansed. Disposable metal forceps maybe required to do this.
10. Assess Skin Tear size, wound and degree of tissue and skin loss. Identify presence and colour of any skin flap. Use the Star Skin Tear Classification System to categorise the type of Skin Tear (**Appendix 1**).
11. If a skin flap is present (category 1A, 1B, 2A and 2B) then realign skin flap using fine metal forceps and/or normal saline moistened cotton buds to gently roll oppose skin edges where possible. Do not cut off skin flap.
12. Redress wound as per nursing care plan below:

Small Skin Tears that are not bleeding or do not have large exudate	Moderate to large size skin tears or a skin tear that is bleeding or has a large exudate
• Cover Skin Tear with soft silicon foam e.g. mepilex foam border, which will secure the skin flap in position. Ensure there is an overlap of at least 3cm onto the surrounding healthy skin. • Mark the skin flap position and size on the outside of the dressing. • Draw an arrow to indicate removal direction so no trauma is caused to healing flap when dressing removal. • Aim to leave initial dressing on for 3-days unless exudate strikes through dressing increase in pain at site or surrounding erythema.	• Primary dressing - Stabilise and secure skin flap by applying a fenestrated sheet of soft silicon mesh e.g. Mepitel over the skin tear. This is a non-adherent silicon dressing and secures the wound edges holding the skin flap in position. • The primary dressing of silicon mesh needs to overlap onto healthy peri wound skin by at least 3cm around entire wound and may remain in situ for 5 days or longer if requested by Wound Management Nurse or Plastic's team. • If wound still bleeding apply a calcium alginate dressing over the primary dressing of silicon mesh to assist with haemostasis e.g. Curasorb. Secondary dressing – cover with an absorbent pad. • Hold dressing in situ with underpadding e.g. kerlix and / or crepe bandage.

13. Dispose of gloves.
14. Dispose of waste.
15. Remove gloves if used, perform hand hygiene.
16. Clean the trolley with a combined detergent/disinfectant wipe.
17. Perform hand hygiene.
18. Document findings and assessments on Wound Management Plan.

6.8 Compression Bandaging

Refer to Medical staff or Wound Management Specialist Nurse for a management plan and how to access the required bandages.

7. Discharge Planning

Where patients require a continuation of management of their skin tear referral to the appropriate clinical services on discharge the following should be performed.

1. Ensure referral and current wound management plan are supplied to the service provider.
2. Provide education and a current wound management plan to the patient and or carer.
3. Provide additional wound supplies on discharge.
4. Ensure the patient/carer are aware of any follow-up appointments and/or service referrals.
5. Provide written and verbal education to the patient/carer regarding adverse signs and symptoms:
 - Infection
 - Wound deterioration
 - Other potential problems, e.g. pain or bleeding

8. Compliance, monitoring and evaluation

- Incidents will be recorded on Datix-Cims and investigated in accordance with service requirements for the management of Datix-CIMS.
- HACs are reported via the Business Intelligence Unit (BIU).
- Clinical Management teams receive data reports for their services.
- The MHPHDS Safety, Quality and Performance is responsible for the auditing and reporting to the MHPHDS Executive ([LINK to SQP Audit tools](#)).

Related internal policies, procedures and guidelines

[SCGOPHCG Infection Prevention and Control Manual - Aseptic Technique Policy](#)
[SCGOPHCG Wound Management Nursing Practice Guideline -NPG16](#)
[SCGH Clinical Photography Policy No.180](#)
[MHPHDS Aseptic Technique Policy](#)
[MHPHDS Hand Hygiene Policy](#)
[Braden Scale – For Predicting Pressure Sore Risk](#)

References

[ACSQHC Fact Sheet Preventing Pressure Injuries and Wound Management](#)
[Infection Prevention and Control in Western Australian Healthcare Facilities](#)
[Australian Guidelines for the Prevention and Control of Infection in Healthcare \(2019\)](#)

Useful resources (including related forms) (if required)

[Prevention and Treatment of Pressure Ulcers/Injuries Clinical Practice Guideline. The International Guideline 2019](#)

Policy Sponsor	MHPHDS Director of Nursing				
Policy Contact	MHPHDS Director of Nursing				
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Approved by:	Director of Nursing			Date:	01/03/2022
Endorsed by:	MHPHDS Director of Clinical Services			Date:	28/04/2022
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☐  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration				
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management ☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry				
Chief Psychiatrist's Standards for Clinical Care:	Care Planning, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Transfer of Care				
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Document Version Control			
Date	Version	Author	Notes
28/04/2022	1.0	A/Policy and Audit Officer	Initial endorsement, this new procedure is to guide to inform staff of wound management and incorporates information from the rescinded MH IPC Wound Care Guideline – A10,

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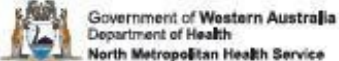
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Appendix 1 - STAR Skin Tear Classification



STAR Skin Tear Classification System Guidelines






1. Control bleeding and clean the wound according to protocol.
2. Realign (if possible) any skin or flap.
3. Assess degree of tissue loss and skin or flap colour using the STAR Classification System.
4. Assess the surrounding skin condition for fragility, swelling, discolouration or bruising.
5. Assess the person, their wound and their healing environment as per protocol.
6. If skin or flap colour is pale, dusky or darkened reassess in 24-48 hours or at the first dressing change.

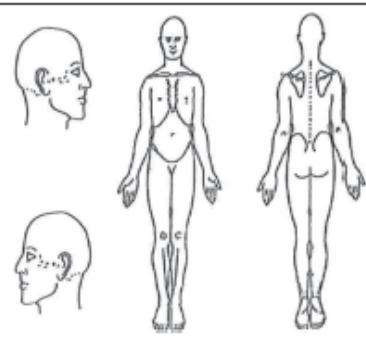
Category 1a A skin tear where the edges can be realigned to the normal anatomical position (without undue stretching) and the skin or flap colour is not pale, dusky or darkened.	
Category 1b A skin tear where the edges can be realigned to the normal anatomical position (without undue stretching) and the skin or flap colour is pale, dusky or darkened.	
Category 2a A skin tear where the edges cannot be realigned to the normal anatomical position and the skin or flap colour is not pale, dusky or darkened.	
Category 2b A skin tear where the edges cannot be realigned to the normal anatomical position and the skin or flap colour is pale, dusky or darkened.	
Category 3 A skin tear where the skin flap is completely absent.	

References:

1 Payne, R., & Martin, M. (1993). Defining and classifying skin tears: Need for a common language ... a critique and revision of the Payne-Martin Classification system for skin tears. *Ostomy Wound Management*, 39(5), 16-20. 2 Photographs courtesy of the Skin Tear Audit Research (STAR) photographic library, Silver Chain Nursing Association and School of Nursing and Midwifery, Curtin University of Technology. 3 Carville, K., Lewin, G., Newall, N., Haskelhurst, P., Michael, R., Santamaría, N., & Roberts, P. (2007). STAR: A consensus for skin tear classification. *Primary Intention*, 15(1), 18-28.

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Appendix 2 – Pressure Injury Risk Management Plan and Braden Scale

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH Hospital PRESSURE INJURY RISK MANAGEMENT PLAN					Surname		Sex	U.R. No
					Forenames		D.O.B.	
					Address			
					Ward	Registrar	Consultant	
Use Patient I.D. Label when available								
RISK ASSESSMENT: Assess for risk on admission to ward using the Braden Scale below. RISK RE-ASSESSMENT: - For patients aged 65 and over, assess every week for 4 consecutive weeks following admission - Following a change in clinical condition e.g. cognitive decline or deterioration in medical condition - Following a change in a patient's mobility - When a patient has prolonged periods of sitting or bed rest - Annually for long stay patients								
BRADEN SCALE (Abridged)								
RISK FACTOR	SCORE				DATE	DATE	DATE	DATE
	4	3	2	1				
Sensory Perception	No Impairment	Slightly Limited	Very Limited	Completely Limited				
Moisture	Rarely Moist	Occasionally Moist	Often Moist	Constantly Moist				
Activity	Walks Frequently	Walks Occasionally	Chairfast	Bedfast				
Mobility	No Limitations	Slightly Limited	Very Limited	Completely Immobile				
Nutrition	Excellent	Adequate	Probably Inadequate	Very Poor				
Friction & Shear		No Apparent Problem	Potential Problem	Problem				
TOTAL SCORE								
SIGNATURE OF ASSESSOR								
DESIGNATION OF ASSESSOR								
Source: Barbara Braden and Nancy Bergstrom. Copyright 1988 (reprinted with permission)								
SCORE 19-23: GENERALLY NOT AT RISK = No further interventions necessary SCORE 6-18: AT RISK = Follow interventions over page								
DOCUMENTATION IF PRESSURE ULCER/S PRESENT ON ADMISSION OR ACQUIRED								
☐ Pressure ulcer documented in patient's medical record ☐ Pressure ulcer alert documented in PSOLIS Care Plan ☐ Pressure ulcer care plan/treatment chart with relevant stage of pressure ulcer ☐ Inform CNS ☐ CIMS Incident Report (only if acquired during admission)								
Date: _____ Signature: _____								
					Indicate location of pressure ulcer/s			

PMR 49 PRESSURE INJURY RISK MANAGEMENT PLAN

NORTH METROPOLITAN HEALTH SERVICE MENTAL HEALTH Hospital PRESSURE INJURY RISK MANAGEMENT PLAN	Surname		Sex	U.R. No
	Forenames		D.O.B.	
	Address			
	Ward	Registrar	Consultant	
	Use Patient I.D. Label when available			
PRESSURE INJURY PREVENTION CARE PLAN Please initial interventions commenced				
SKIN CARE		DATE		
Daily skin assessment				
Gently apply moisturiser to dry, flaky or scaling skin. Do not rub skin.				
Use soap free, pH neutral (pH 4-6) skin cleansers, avoid soap.				
If patient is incontinent of urine or faeces, assess and treat cause AND:				
- Use continence pads / protective sheets that are quick drying				
- Use skin barrier after cleaning soiled area				
NUTRITION				
Encourage adequate hydration and balanced diet				
Refer to dietician if Braden Scale nutrition score is less than 3				
Implement nutrition supplement chart if clinically indicated by Dietician				
POSITIONING AND REPOSITIONING				
If patient has diminished mobility or activity, institute a turning schedule on re-positioning chart				
Specify frequency: minutes				
Use pillows/foam wedges to avoid direct contact between bony prominences, ensuring body alignment is maintained				
When sitting, ensure patient foot placement is below the level of hips with optimum 90° hip and knee flexion (unless contraindicated)				
If existing pressure ulcer on buttocks / sacrum:				
Limit periods of sitting out of bed to minutes				
ACTIVITY AND MOBILITY				
Encourage patient to maximise their activity and mobilisation consistent with their medical condition, ability and energy levels				
Referral to Physiotherapist				
SUPPORT SURFACES				
Consider static and/or dynamic devices on beds and chairs e.g. alternating air mattress				
Liaise with Occupational Therapist and/or Physiotherapist for seating and bedding needs				
FRICTION AND SHEAR				
If friction and/or shear are evident:				
Use mechanical lifting aids: ☐ Hoist ☐ Slide Sheet				
Elevate foot of bed 10°- 20° to reduce sliding (unless contraindicated)				
Lower head of bed to <30° to reduce sliding (unless contraindicated)				
HEELS				
If heel/s vulnerable or exposed to unrelieved pressure				
Relieve pressure from heels with pillows or foam heel elevation device				
Use bed cradle to prevent constriction of the feet and toes by bed linen				
Referral to Podiatrist if adapted or off-loading footwear is required				
PATIENT AND CARER EDUCATION				
Education regarding risk factors for developing pressure ulcers, strategies for prevention and the importance of skin inspection and care				
Verbal education				
Written information given e.g. Prevention of Pressure Ulcers booklet				
DOCUMENTATION				
Notify CNS if Braden Score is 6-18				
Document Braden Score and risk level in patient's progress notes				
Document risk in PSOLIS Management Plan				

BRADEN SCALE – For Predicting Pressure Sore Risk

SEVERE RISK: Total score ≤ 9 HIGH RISK: Total score 10-12					DATE OF ASSESS →			
MODERATE RISK: Total score 13-14 MILD RISK: Total score 15-18								
RISK FACTOR	SCORE/DESCRIPTION				1	2	3	4
SENSORY PERCEPTION Ability to respond meaningfully to pressure-related discomfort	1. COMPLETELY LIMITED – Unresponsive (does not moan, flinch, or grasp) to painful stimuli, due to diminished level of consciousness or sedation, OR limited ability to feel pain over most of body surface.	2. VERY LIMITED – Responds only to painful stimuli. Cannot communicate discomfort except by moaning or restlessness, OR has a sensory impairment which limits the ability to feel pain or discomfort over ½ of body.	3. SLIGHTLY LIMITED – Responds to verbal commands but cannot always communicate discomfort or need to be turned, OR has some sensory impairment which limits ability to feel pain or discomfort in 1 or 2 extremities.	4. NO IMPAIRMENT – Responds to verbal commands. Has no sensory deficit which would limit ability to feel or voice pain or discomfort.				
MOISTURE Degree to which skin is exposed to moisture	1. CONSTANTLY MOIST – Skin is kept moist almost constantly by perspiration, urine, etc. Dampness is detected every time patient is moved or turned.	2. OFTEN MOIST – Skin is often but not always moist. Linen must be changed at least once a shift.	3. OCCASIONALLY MOIST – Skin is occasionally moist, requiring an extra linen change approximately once a day.	4. RARELY MOIST – Skin is usually dry; linen only requires changing at routine intervals.				
ACTIVITY Degree of physical activity	1. BEDFAST – Confined to bed.	2. CHAIRFAST – Ability to walk severely limited or nonexistent. Cannot bear own weight and/or must be assisted into chair or wheelchair.	3. WALKS OCCASIONALLY – Walks occasionally during day, but for very short distances, with or without assistance. Spends majority of each shift in bed or chair.	4. WALKS FREQUENTLY – Walks outside the room at least twice a day and inside room at least once every 2 hours during waking hours.				
MOBILITY Ability to change and control body position	1. COMPLETELY IMMOBILE – Does not make even slight changes in body or extremity position without assistance.	2. VERY LIMITED – Makes occasional slight changes in body or extremity position but unable to make frequent or significant changes independently.	3. SLIGHTLY LIMITED – Makes frequent though slight changes in body or extremity position independently.	4. NO LIMITATIONS – Makes major and frequent changes in position without assistance.				
NUTRITION Usual food intake pattern ¹ NPO: Nothing by mouth. ² IV: Intravenously. ³ TPN: Total parenteral nutrition.	1. VERY POOR – Never eats a complete meal. Rarely eats more than 1/3 of any food offered. Eats 2 servings or less of protein (meat or dairy products) per day. Takes fluids poorly. Does not take a liquid dietary supplement, OR is NPO ¹ and/or maintained on clear liquids or IV ² for more than 3 days.	2. PROBABLY INADEQUATE – Rarely eats a complete meal and generally eats only about ½ of any food offered. Protein intake includes only 3 servings of meat or dairy products per day. Occasionally will take a dietary supplement OR receives less than optimum amount of liquid diet or tube feeding.	3. ADEQUATE – Eats over half of most meals. Eats a total of 4 servings of protein (meat, dairy products) each day. Occasionally refuses a meal, but will usually take a supplement if offered, OR is on a tube feeding or TPN ³ regimen, which probably meets most of nutritional needs.	4. EXCELLENT – Eats most of every meal. Never refuses a meal. Usually eats a total of 4 or more servings of meat and dairy products. Occasionally eats between meals. Does not require supplementation.				
FRICTION AND SHEAR	1. PROBLEM – Requires moderate to maximum assistance in moving. Complete lifting without sliding against sheets is impossible. Frequently slides down in bed or chair, requiring frequent repositioning with maximum assistance. Spasticity, contractures, or agitation leads to almost constant friction.	2. POTENTIAL PROBLEM – Moves feebly or requires minimum assistance. During a move, skin probably slides to some extent against sheets, chair, restraints, or other devices. Maintains relatively good position in chair or bed most of the time but occasionally slides down.	3. NO APPARENT PROBLEM – Moves in bed and in chair independently and has sufficient muscle strength to lift up completely during move. Maintains good position in bed or chair at all times.					
TOTAL SCORE	Total score of 12 or less represents HIGH RISK							
ASSESS	DATE	EVALUATOR SIGNATURE/TITLE		ASSESS.	DATE	EVALUATOR SIGNATURE/TITLE		
1	/ /			3	/ /			
2	/ /			4	/ /			
NAME-Last		First	Middle	Attending Physician		Record No.	Room/Bed	

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BRADEN SCALE

Appendix 3 – Abbey Pain Assessment Scale (Follow on Assessment)

ABBEY PAIN ASSESSMENT SCALE (FOLLOW ON ASSESSMENT)

DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME	DATE AND TIME
VOCALISATION eg: whimpering, groaning, crying <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
FACIAL EXPRESSION eg: looking tense, frowning grimacing, looking frightened <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
CHANGE IN BODY LANGUAGE eg: fidgeting, rocking, guarding part of body, withdrawn <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
BEHAVIOURAL CHANGE eg: increased confusion, refusing to eat, alteration in usual patterns <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
PHYSIOLOGICAL CHANGES eg: temperature, pulse or blood pressure outside normal limits, perspiring, flushing or pallor <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
PHYSICAL CHANGES eg: skin tears, pressure areas, arthritis, contractures, previous injuries <i>Absent 0 Mild 1 Moderate 2 Severe 3</i>										
Total score =										
Signature of person completing score										
0-2 NO PAIN	3-7 MILD PAIN			8-13 MODERATE PAIN				14 + SEVERE		

The Pain Scale is an instrument designed to assist in the assessment of pain in patients who are unable to clearly articulate their needs.

The Abbey pain scale does not differentiate between distress and pain, therefore measuring the effectiveness of pain relieving interventions is essential.

The pain scale should be used as a **movement based assessment**, therefore observe the patient while they are being moved, during pressure area care, while showering etc.

A second evaluation should be conducted **1 hour after** any intervention taken. If, at this assessment, the score on the pain scale is the same, or worse, consider further intervention and act as appropriate.

Complete the scale hourly until the patient scores mild pain then 4 hourly for 24 hours treating pain if it recurs.

If the pain/distress persists, undertake a comprehensive assessment of all facets of the patients care and monitor closely over 24 hours including further intervention undertaken.

If there is no improvement during that time notify the doctor/pain team of the pain scores and actions taken.

Addressograph

Abbey Pain Scale
For measurement of pain in patients who cannot verbalise.

Name and designation of person completing the scale:

Date: Time:

How to use scale: While observing the patient, score questions 1 to 6

Q1. Vocalisation

eg. whimpering, groaning, crying

Absent 0 Mild 1 Moderate 2 Severe 3

Q1

Q2. Facial expression

eg: looking tense, frowning, grimacing, looking frightened

Absent 0 Mild 1 Moderate 2 Severe 3

Q2

Q3. Change in body language

eg: fidgeting, rocking, guarding part of body, withdrawn

Absent 0 Mild 1 Moderate 2 Severe 3

Q3

Q4. Behavioural Change

eg: increased confusion, refusing to eat, alteration in usual patterns

Absent 0 Mild 1 Moderate 2 Severe 3

Q4

Q5. Physiological change

eg: temperature, pulse or blood pressure outside normal limits,
perspiring, flushing or pallor

Absent 0 Mild 1 Moderate 2 Severe 3

Q5

Q6. Physical changes

eg: skin tears, pressure areas, arthritis, contractures, previous injuries.

Absent 0 Mild 1 Moderate 2 Severe 3

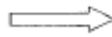
Q6

Add scores for 1 - 6 and record here



Total Pain Score

Now tick the box that matches the
Total Pain Score



0 - 2 No pain	3 - 7 Mild	8 - 13 Moderate	14 + Severe
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Finally, tick the box which matches
the type of pain



Chronic	Acute	Acute on Chronic
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Abbey, J, De Bellis, A, Piller, N, Esterman, A, Giles, L, Parker, D and Lowday, B.
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